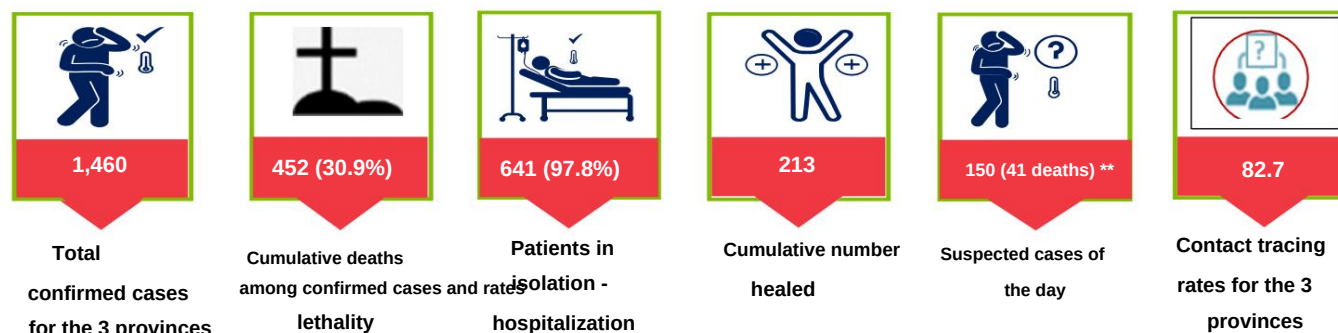


**Public Health Emergency Operations Center
"COUSP-DRC"**
MVE-17 Incident Management System
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC
SitRep N°48/MVB_01/07/2026

Provinces affected (3)	Ituri, North Kivu, South Kivu
Health Zones (ZS) affected (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 1, 2026
Publication date	July 2, 2026



* Data updated and subject to change following the cleaning and reclassification process and harmonization of the DHIS2 database, including the consolidation of information from health zones, laboratories and case management structures.

**Suspicious deaths occurring in the community are subject to a thorough investigation with a view to possible reclassification (Non-cases, probable cases).

1. HIGHLIGHTS

Fifty - four (54) new confirmed cases, including 9 deaths, were reported on July 1, 2026, in the provinces of Ituri (50 cases, including 9 deaths) and North Kivu (4 cases). These confirmed cases originate from the health zones of **Bunia (29), Rwampara (7), Mongbwalu (6), Nizi (4), Lita (1), and Komanda (1)** in Ituri, and **Katwa (2) and Beni (2)** in North Kivu.

- **Fourteen (14)** deaths were recorded among patients with Ebola virus disease (EVD) in Ebola treatment centers (ETCs) in Ituri province. Five reported deaths were distributed as follows: (3 at the Bunia ETC, 1 at the Mongbwalu ETC and 1 at the Rwampara ETC) and 9 community deaths.
- **Twenty-one (21)** patients with Ebola virus disease, including 19 in Ituri province (7 at CTE Elikya, 6 at CTE Mongbwalu, 4 at CTE ISTM and 2 at CTE CME) and 2 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.
- **Security incident at PK51 in the Nia-Nia security zone in Ituri.**

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the **Democratic Republic of Congo** (DRC), shares a long and porous border with **Uganda** and **South Sudan**. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflicts and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Territory of Djugu, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola outbreak which was then raging in North Kivu.

The current epidemic, due to Bundibugyo Ebolavirus, was officially declared on May 15, 2026 by the Minister of Public Health.

The three affected provinces have a total estimated population of nearly 15 million, with internal displacement massive and cross-border flows towards neighbouring countries.

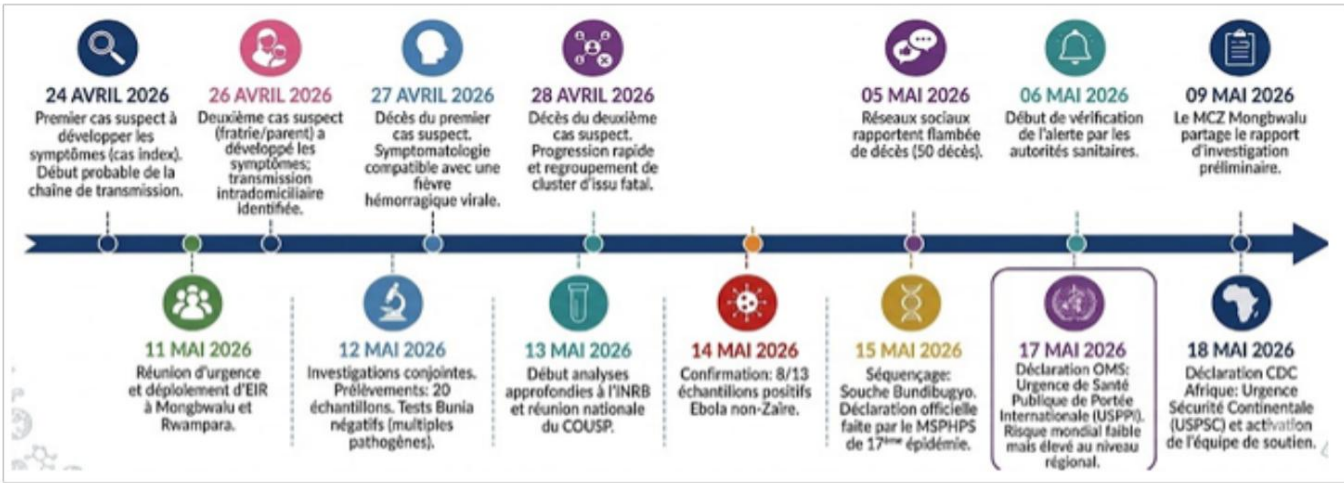


Figure 1: Chronology of events in the Ebola-Bundibugyo epidemic

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1. Spatial distribution and active hotspots as of July 1, 2026

Table 1: Distribution of confirmed cases and deaths by affected province as of July 1, 2026.

Province	Confirmed cases	Deaths confirmed	CFR	Affected health zones	New cases (30 June 2026)
Ituri	1,333 28.5%	124 57.3%	33.3% 1,460 30.9%	24 out of 36 (66.7%)	50
North Kivu	Five (5) deaths in Ituri and 5 deaths in North Kivu were recorded among patients with EVD (previous confirmed cases) in ETD/			11 out of 34 (32.3%)	4
South Kivu				1 out of 34 (2.9%)	0
Total				36 out of 104 (34.6%)	54

CT.

At the end of the day on July 1, 2026, no new health zones were affected. The number of affected health zones remains at 36/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

Distribution by affected zones

- **Ituri Province (1,333 cases):** Bunia (416), Rwampara (308), Mongbwalu (270), Nyankunde (95), Nizi (65), Lita (32), Mangala (24), Komanda (19), Bambu (16), Tchomia (14), Nia-Nia (12), Kilo (9), Logo (7), Aungba (6), Damas (5), Rimba (3), Aru (3), Drodoro (3), Kambala (2), Mambasa (2), Mandima (2), Gety (1), Fataki (1), and Lolwa (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (124 cases):** Katwa (49), Butembo (34), Beni (26), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (3), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.2. Time Analysis

We are observing an increasing number of confirmed cases from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) has contributed to early detection, confirming continued and increased community transmission in week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 represent the peak so far, with more than 300 cases per week.

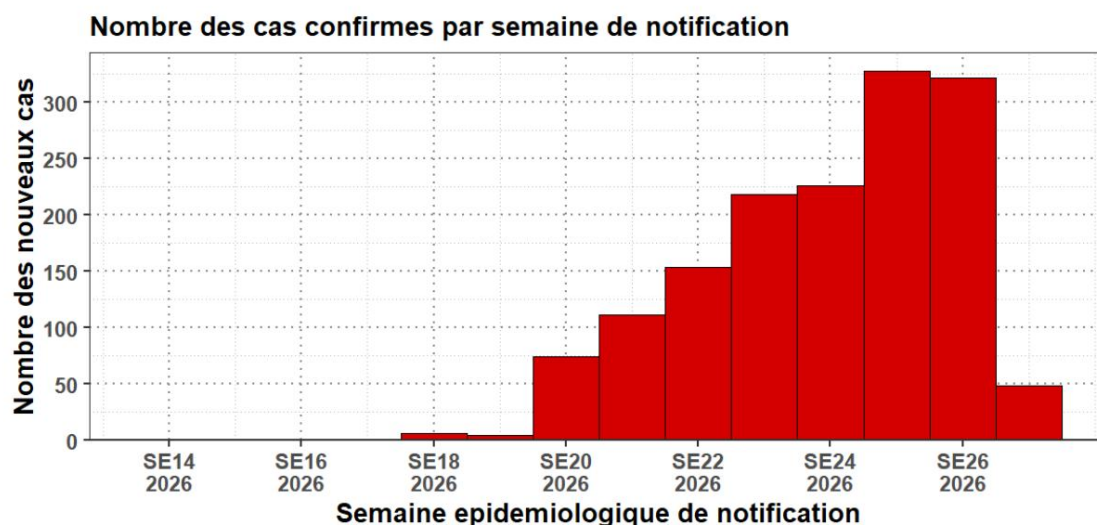


Figure 2: Confirmed cases per epidemiological week of symptom onset

We are observing an increasing number of confirmed cases from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) has contributed to early detection, confirming continued and increased community transmission in week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, week 25 represents the peak so far, with more than 300 cases per week.

3.3. Epidemic mapping

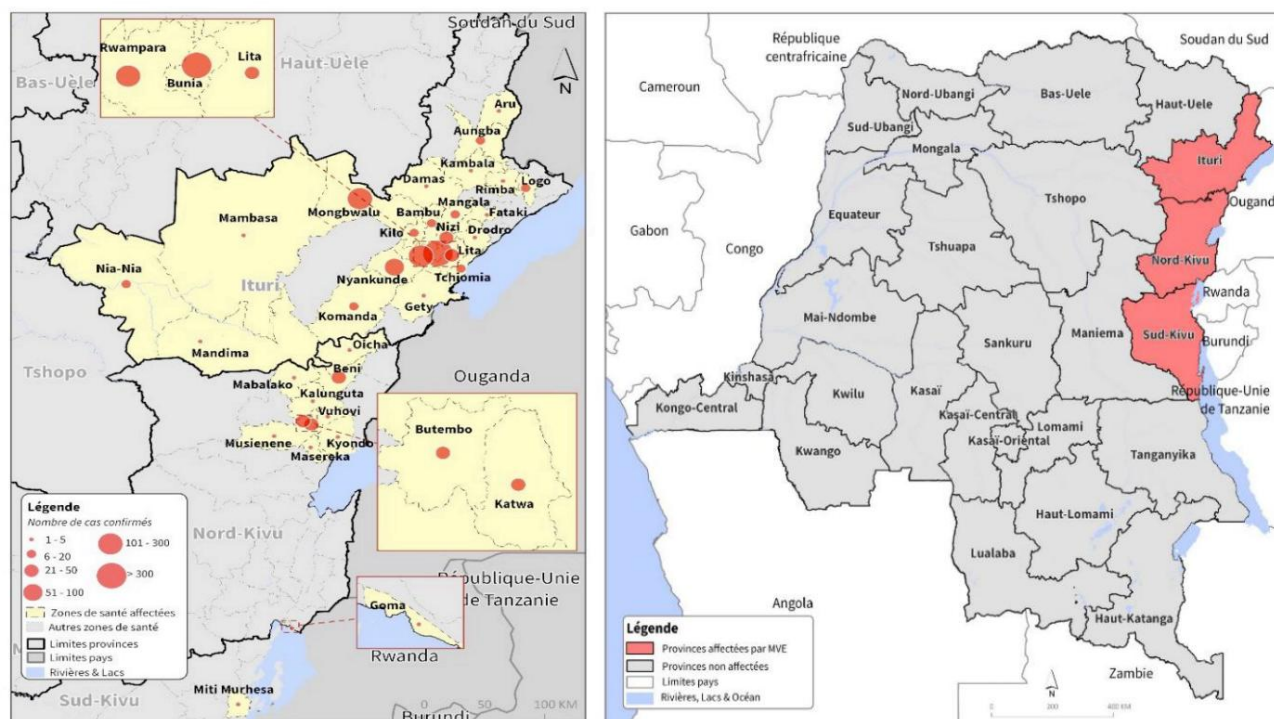


Figure 3: Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 1, 2026.

Table 2. Distribution of confirmed Ebola virus disease cases and deaths by province and health zone (July 1, 2026)

Health Zone	Cumulative confirmed cases:	Cumulative confirmed deaths :	Case fatality rate (CFR%)
ITURI	1333,		28.5%
Bunia	416,		24.8%
Rwampara	308,		20.5%
Mongbwalu	270,		47.0%
Nyankunde	95,		16.8%
Nizi	65,		29.2%
Lita	32,		34.4%
Komanda	19,		26.3%
Bamboo	16,		31.3%
Kilo			11.1%
Mangala	9,		50.0%
Tchomia	24,		14.3%
Damascus			0.0%
Aungba			33.3%
Rimba			0.0%
Aru			33.3%
Mambasa	14, 5, 6, 3, 3, 2	380,103,631,271,619,115,511,122,020,111	50.0%
Logo	7	0	0.0%
Drodro	3	3	100.0%
Gety	1	0	0.0%

Health Zone	Cumulative confirmed cases	Total confirmed deaths	Case fatality rate (CFR%)
ITURI	1333	380 2	28.5%
Kambala	2	5	100.0%
Nia-Nia*	12	0	41.7%
Fataki		1	0.0%
Mandima	1		50.0%
Lolwa			100.0%
Other areas not Again identified	2 1 17	1 0	0.0%
NORTH KIVU	124	71	57.3%
Butembo	34	15	44.1%
Katwa	49	33	67.3%
Blessed	26	15	57.7%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3	1	66.7%
Goma		2	0.0%
Masereka		0	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene	1 1 1 1 3	1 0 2	66.7%
SOUTH KIVU	3	1	33.3%
Miti Murhesa			33.3%
TOTAL	31460		30.9%

1,452 The 5 imported cases in Wamba (Haut Uélé Province) are counted in Niania and have been returned to Niania.

In the past 24 hours, 54 new confirmed cases, including 9 deaths, have been recorded in the provinces of Ituri (50 cases, including 9 deaths) and North Kivu (4 cases), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,460 confirmed cases and 452 deaths, expressing an overall case fatality rate of 30.9%.

a. Provincial dynamics and concentration of cases

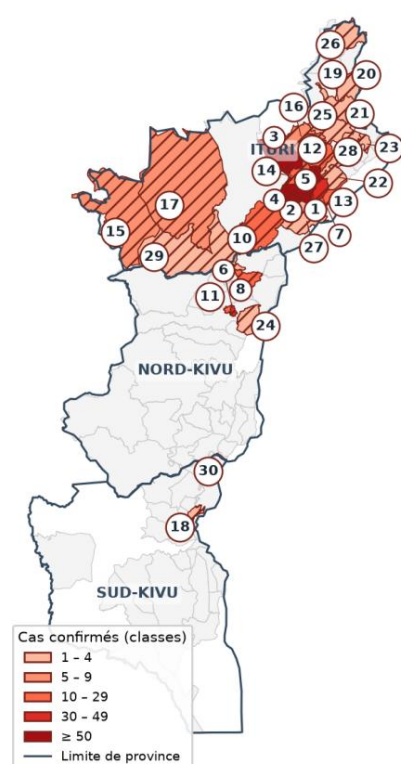
The **Ituri** province alone accounts for **91.3%** of cases (n=1,333) and **84.1%** of deaths (n=380), representing a case fatality rate of **28.5%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (416 cases), Rwampara (308 cases), and Mongbwalu (270 cases) remain the most affected, accounting for approximately 74.6% of reported cases at the provincial level and 68.1% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.5%) and Bunia (24.8%), while the **Mongbwalu** zone also has a high case fatality rate of **47.0%** (127 deaths out of 270 cases), suggesting persistent challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

b. Severity of the epidemic in North Kivu

A total of 124 confirmed cases and 71 deaths have been reported in North Kivu province since the start of the epidemic, representing a **case fatality rate of 57.3%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the **Katwa** health zones.

(67.3%), **Oicha** (66.7%), and **Beni** (57.7%). The Katwa health zone had the highest number of reported cases (n=49), followed by the Butembo health zone (n=34) with a case fatality rate of 44.1%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

Cas confirmés MVE par zone de santé



Zones de santé touchées

1	Bunia (389)
2	Rwampara (317)
3	Mongwalu (260)
4	Nyankunde (90)
5	Nizi (68)
6	Katwa (46)
7	Lita (37)
8	Beni (25)
9	Mangala (25)
10	Komanda (20)
11	Butembo (19)
12	Bambu (18)
13	Tchomia (16)
14	Kilo (11)
15	Nia Nia (8)
16	Damas (6)
17	Mambasa (5)
18	Miti Murhesa (4)
19	Aungba (4)
20	Aru (4)
21	Rimba (3)
22	Drodoro (3)
23	Logo (2)
24	Kyondo (2)
25	Kambala (1)
26	Adi (1)
27	Gety (1)
28	Fataki (1)
29	Mandima (1)
30	Goma (1)

Figure 3: Spatial distribution of confirmed Ebola cases by health zone

4. PILLAR-BASED RESPONSE ACTIONS

4.1. Coordination

National / Ituri:

- Field visit by the Director General of the INSP with the coordination team to the health zones of Nizi and Nyankunde for assess the land that will be used for the construction of the CTE.
- Identification of the site for the construction of the new CTE in the Bunia ZS.

4.2. Epidemiological surveillance and investigation

Table 1: Monitoring Indicators (July 1, 2026)

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		71	0	2	73
New Alerts		155	723	7	885
Total alerts for the day		226	723	9	958
Alerts investigated		110	723	9	842
Investigation rate		48.7%	100.0%	100.0%	87.9%
Alerts confirmed	Alive	82	25	2	109
	Deceased	28	13	0	41
Total suspected cases for today		110	38	2	150

At the end of the day on July 1, 2026, **958 alerts** were reported across the three provinces, **842 (87.9%)** were investigated, **150 suspected cases including 41 deaths** were identified (Table 3).

Table 2: Contact tracing of confirmed cases as of July 1, 2026

Province	Contacts under follow-up	Contacts seen (n)	Follow-up rate (%)
Ituri*	(n) 8	6,973	83.2%
North Kivu*	376 2	1,981	81.0%
Total	445 10 821	8,954	82.7%

* The follow-up rate was calculated only for health zones that reported (completeness: 93.3%). The Logo and Nia-Nia health zones in Ituri did not have not data available at the time the reports were consolidated.

- Continued in-depth investigations, exhaustive listing, and systematic monitoring of contacts around the cases confirmed in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **220 new contacts** were listed, including **89** in 4 health zones in Ituri province and **131** in 3 health zones in the province of North Kivu.
- **16 contacts** have **become suspects**, including **9** in Ituri province and **7** in North Kivu province.
- **204 contacts successfully completed their 21-day follow-up**, including **89** in 3 health zones in Ituri province and **115** in 3 health zones in North Kivu province.
- Contact tracing of the confirmed case in France continues in Kinshasa, with a follow-up rate of 67.3% (72 contacts seen out of 107 listed), and they are now on day 10 of monitoring. Three contacts became suspected cases on day 8; two have been isolated with an initial negative test, while the third is not very cooperative.
- Investigations into alerts and suspected cases are continuing in Kinshasa.

4.3. Monitoring Point on checkpoint and point of entry (PoC/PoE) activities

- Briefing of PoE/PoC providers on DHIS2.
- **Ituri** : Five (5) alerts were notified to the PoCs, of which 3 were live, 1 was validated and referred to a CTE and the other 2 were invalidated; 2 lifeless bodies were intercepted after verification of the document by the EDS teams, the Swab was carried out and then released for burial.

Table 3: Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 1, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55) 74	100.0% (14/14) 98.5% (68/69)	
Number of people who went through PoE/PoC testing,	710	52 471 357 189	
% of travelers screened, % of	99.3%	97.1% 98.2%	
travelers who washed their hands, % of	92.9%	97.1% 95%	
travelers made aware of the issue	93%	97.1% 95.1%	
Number of alerts notified		0 5	
Number of validated live alerts		0 2	
Number of lifeless bodies intercepted today		1 3	
Number of problematic contacts intercepted today; % of		0 0	
validated live alerts referred to CT/CTE; % of deceased	5 2 2 0	0.0% 0.0%	
bodies swabbed	33.3%	100.0% 75.0%	
Number of positive results of referred suspected cases	50.0% 0	0 0	

4.4. Laboratory

Table 4: Laboratory indicators (July 1, 2026)

Indicator	Ituri	North Kivu	South Kivu
Samples collected	132	96	2
Samples analyzed Positive	132	96	0
samples	50	4	-
Positivity rate	37.9%	4.2%	-
Samples currently being analyzed			2
Positive samples (suspected cases who died)	0 0	0 0	0

4.5. Research

Launch of the clinical trial in CTE/CME

4.6. Holistic care

Ituri

- **Five (5) deaths** were recorded among patients with Ebola virus disease in Ebola treatment centers in Ituri province (3 to CTE of Bunia, 1 at the CTE of Mongbwalu and 1 at the CTE Rwampara.
- **Twenty-one (21)** patients with Ebola virus disease, including 19 in Ituri province (7 at CTE Elikya, 6 at CTE Mongbwalu, 4 at CTE ISTM and 2 at CTE CME) and 2 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.
- At the end of the day on July 1st, 805 hot meals were served, including 150 to confirmed cases, 179 to suspected cases, 447 to PPLs and 29 to accompanying persons. Twenty (3) dry rations were also distributed.

North Kivu

- The child registered at CT Musienene as a suspected case and suspected of being the positive case who escaped from CH Mutiri was released after two negative tests; Training of 51 providers from CTE Beni with support from IMC; Participation in the briefing of supervisors on IPC with support from IMA; Supervision of the distribution of dry rations to the 3 providers and one APA from the NZUMA district.
- Distribution of dry rations to service providers and local authorities forced to flee their homes following threats made by the family of the deceased confirmed in the CTE Beni, after receiving initial treatment at the Divine PS.

Table 5: Key monitoring indicators as of July 1, 2026

Province	Zone de sante	Cas confirmés	Deces confirmés - statut final	Taux de létalité (Communautaire +intra Hosp)	Deces a la notification	Cas confirmés notifiés dans 24h	Deces confirmés parmi les notifiés dans 24h	Taux de létalité parmi les cas confirmés notifiés en 24h de DDS	Décès communautaire : Décès à la notification + Décès en 24h de notification	Taux de létalité (décès communautaire)	Cas confirmés notifiés apres 24h	Deces confirmés parmi les notifiés apres 24h	Taux de létalité parmi les cas confirmés notifiés après 24h de DDS
Ituri	Bunia	430	82	19.1%	80	108	20	18.5%	100	23.3%	282	53	18.8%
Ituri	Rwampara	326	59	18.1%	41	107	22	20.6%	63	19.3%	177	27	15.3%
Ituri	Mongbwalu	261	95	36.4%	95	60	24	40.0%	119	45.6%	189	66	34.9%
Ituri	Nyankunde	94	7	7.4%	6	34	2	5.9%	8	8.5%	57	5	8.8%
Ituri	Nizi	93	31	33.3%	30	19	9	47.4%	39	41.9%	47	11	23.4%
Ituri	Lita	41	16	39.0%	16	10	3	30.0%	19	46.3%	24	9	37.5%
Ituri	Mangala	30	12	40.0%	12	8	4	50.0%	16	53.3%	7	1	14.3%
Ituri	Komanda	22	6	27.3%	6	6	1	16.7%	7	31.8%	11	4	36.4%
Ituri	Bambu	19	3	15.8%	3	7	0	0.0%	3	15.8%	11	2	18.2%
Ituri	Tchomia	18	8	44.4%	4	4	1	25.0%	5	27.8%	13	7	53.8%
Ituri	Kilo	12	2	16.7%	2	3	0	0.0%	2	16.7%	7	2	28.6%
Ituri	Nia Nia	9	3	33.3%	3	2	0	0.0%	3	33.3%	5	2	40.0%
Ituri	Damas	7	2	28.6%	2	1	0	0.0%	2	28.6%	4	1	25.0%
Ituri	Aungba	5	2	40.0%	2	3	1	33.3%	3	60.0%	1	0	0.0%
Ituri	Mambasa	5	3	60.0%	3	1	0	0.0%	3	60.0%	1	0	0.0%
Ituri	Aru	4	1	25.0%	1	2	1	50.0%	2	50.0%	1	0	0.0%
Ituri	Mandima	4	1	25.0%	1	0	0	0.0%	1	25.0%	3	1	33.3%
Ituri	Drodro	3	3	100.0%	3	0	0	0.0%	3	100.0%	3	3	100.0%
Ituri	Rimba	3	0	0.0%	0	0	0	0.0%	0	0.0%	3	0	0.0%
Ituri	Logo	2	0	0.0%	0	1	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Adi	1	1	100.0%	1	0	0	0.0%	1	100.0%	0	0	0.0%
Ituri	Fataki	1	0	0.0%	0	0	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Gety	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Kambala	1	0	0.0%	0	0	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Lohwa	1	1	100.0%	1	0	0	0.0%	1	100.0%	1	1	100.0%
Nord Kivu	Katwa	49	26	53.1%	26	40	23	57.5%	49	100.0%	6	2	33.3%
Nord Kivu	Beni	24	17	70.8%	12	5	5	100.0%	17	70.8%	15	9	60.0%
Nord Kivu	Butembo	19	9	47.4%	8	3	0	0.0%	8	42.1%	16	9	56.3%
Nord Kivu	Kyondo	2	1	50.0%	0	0	0	0.0%	0	0.0%	2	1	50.0%
Nord Kivu	Goma	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Sud Kivu	Miti Murhesa	3	1	33.3%	1	0	0	0.0%	1	33.3%	1	1	100.0%
TOTAL	TOTAL	1,491	392	26.3%	359	426	116	27.2%	475	31.9%	889	217	24.4%

A total of 392 deaths were recorded, the majority (359) of which occurred within the community (community deaths). Thus, the overall community case fatality rate reached 31.9%. The geographical distribution shows a high concentration in Mongbwalu (119 deaths), Bunia (100 deaths), and Katwa (49 deaths). Patients either die within the community or present themselves at health facilities far too late. Late consultations at healthcare facilities and isolation within the community contribute to the problem.

This significantly increases the risk of secondary transmission within households and among patients' contacts. The in-hospital case fatality rate is slightly lower (24.4%), representing 217 deaths.

4.7. Infection Prevention and Control (IPC)

Ituri: 21/27 households were identified and decontaminated around confirmed cases; 55 means of transport (ambulances, hearses) were decontaminated and 33 service providers were briefed on Ebola Virus Disease (EVD); 3 health establishments (HEs) were identified and decontaminated around confirmed cases in the health zones of Bunia, Rwampara, Mongbwalu, and Komanda; 9 public and specific places were decontaminated: office 1; 26/29 health assessments (HAS) were carried out with 2 postponements; IPC/WASH assessments were conducted on 9 HEs in the health zones of Mongbwalu, Bunia, Lita, Nyankunde, and Rwampara; 29 HEs were monitored and supported: Salama Clinic 87.1%; St. Raphael Medical Center 17.4%; St. Pierre Medical Center 58%; Umoja Health Center 35.5%; Bon Berger Clinic 50.2%. PS rapha 50%, PS Healing 39%, PS Maman mwilu 40%; CM Ave maria 30%, CS Poli de frontiere 41.9%; CS Nyakero 40%, CS Lengabo 68.2%; PS Tuluza 25.3%; CM Bethel 22.6%, PS Wisdom 8.1%, PS Benjamin 14.5%, CM Karibuni, 35.5%, CM Spaec. Atlas35.5%; CS Lengabo 68.2%; CS Bigo 59.6%; CM rescue 25.3%, CM Gracia Katsina 9.7%.

North Kivu : Conclusion of the briefing for 21 managers, including EPP (Emergency Preparedness Program) staff and supervisors of the IPC (Integrated Care Pathway) pillar, on the methodology for IPC WASH assessments using the updated scorecard tool and basic database management for KPI indicators, with IMA (International Mutual Aid) support, in preparation for IPC scorecard assessments in 160 health facilities across 10 health zones in North Kivu; 140 facilities were assessed with an average score of 48.7%. Only 2 facilities exceeded a score of 80%: Katwa General Referral Hospital and Kyangike Health Center; 69% of people with primary AIDS (PPA) were assessed as high risk (74/108); 37 blood donations were collected, and 24 bodies were swabbed, including 1 in the Oicha health zone.

4.8. Risk Communication and Community Engagement (RCCE)

Ituri: Two (2) meetings were organized for 8 participants to obtain community commitment to fight against EVD and for their adherence to the commitment and fight against EVD; 86 social mobilization activities (community dialogues and mass awareness) reaching 14,930 (6,250 men and 8,680 women) on EVD preventive measures, community-based surveillance, care, decontamination and EDS in the health zones of Mongbwalu, Nyankunde, Nizi, Rwampara, Nia-Nia, Aungba, Bambu, Aru; 194 educational talks and home visits organized by 200 Recos, 1,252 households were reached and 3,463 people reached (2,318 men and 3,100 women) in the health zones of Bunia; Briefing of 179 providers on community-based surveillance, CREC with practical exercises on the organisation of home visits, facilitation of educational talks in their respective environments and the use of tools (community definition of cases, contact registration form, contact tracing form, and home visit form); 18 broadcasts of spots and messages on preventive measures for Ebola Virus Disease.

Community feedback: hesitancy to go to health facilities for fear of being mistaken by patients for Ebola virus disease; poor nutritional and medical care at the Ebola Treatment Center; discrimination in medical care at the Ebola Treatment Center; poor follow-up of patients in isolation; what measures to take to avoid mass contamination at the livestock market where people are too crowded.

Support for the pillars of the response :

Surveillance: Around the 12 cases of Death and 22 cases of illness, the CREC team supported the Surveillance teams in the different AS in particular the AS of KIGONZE. support in the follow-up of 73 contacts including 12 in Goli, 20 in Dele, 30 in Kisoke and 11 at the Tsere site.

PCI: Support for EDS and PCI teams in the Nyakunde Health Zone.

North Kivu: Twenty (20) local chiefs and leaders on the engagement and mobilization of the respective communities on the respect of barrier measures and the prevention of community incidents in the AS Mukulya; organisation of 2 Community dialogue sessions bringing together 55 APA and Community leaders on the prevention against Ebola in the AS Mateto and Aloya in the ZS of Biena.

4.9. Logistics and Supply

Ituri

26 vehicles and 4 ambulances (PNC, FARDC, HGR Bunia and CTE Rwampara) were made available to the different pillars to ensure their mobility to the axes.

Delivery of medical and communication supplies to Bunia Central Prison, Nizi Pillar PSEA Health Zone and CTE , at the WHO office, at CME (3 x 21kg cylinders).

North Kivu

Receipt of the 2nd batch of medicines / donation from UG PDSS; Delivery of 24 body bags to the EDS pillar, including 14 for adults and 10 for children.

Supply of 10 PPL Kits to the Kalunguta Health Zone and 20 packages of medicines/donation from UG PDSS/World Bank.

Delivery of PCI inputs to the Musienene Health Zone (20 PPL Kits)

Shipment of medicines/donation from UG PDSS to the health zones of Katwa (130 packages) and Butembo (295 packages)

Deployment of 4 tents with accessories from the WFP warehouse to the DPS Bunia office.

4.10. Safety and accessibility

The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.11. Mental health activities and psychosocial support Table 6: Key indicators of mental health activities and psychosocial support as of 1 July 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who have received interventions SMSPS	14 (58.3%)	0	0
Confirmed cases currently being monitored that have benefited from SMSPS interventions	67 (64.4%)	1 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	7 (43.8%)	4 (100.0%)	0
Suspected cases under follow-up that have benefited from SMSPS interventions	60 (56.6%)	30 (100.0%)	2 (100.0%)
Laboratory results have been announced.	30	19	0
Including positive ones	15		0
Number of children separated in daycare who benefited from SMSPS interventions			0
Number of children separated in the community who have benefited from interventions SMSPS	0 1	2 0 0	14
Number of accompanying persons at the CTE who benefited from SMSPS interventions	85	8	0
Number of PPLs who benefited from SMSPS interventions	18	7	2
Number of household and community members who benefited from SMSPS interventions	5	2	0
Number of EDS supported	13	4	0

4.12. Continuity of care - Continued

care for non-cases (children, mothers and other vulnerable individuals) within the continuity of care actions services

- In total, **102 PPLs** are confirmed, including 25 deaths (24.5%) and 36 recoveries.

4.13. Monitoring and evaluation

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

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Challenge No.	Impact	Actions required
1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Performance of the monitoring of contacts not optimal (81.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts

5	confirmed cases not yet broken down by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct data reconciliation, remove potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6	Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 HCW cases including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8	Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10.	Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PERSPECTIVES AND PRIORITY ACTIONS

- Reinforce intervention teams in the affected areas.
- Make means of transport available to field teams.
- Continue listing and tracking contacts in all ZS.

7. ANNEXES



Briefing of PoE/PoC providers on DHIS2



Security incident at PK51 in the Nia-Nia security zone in Ituri

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Avec l'appui technique de l'Organisation Mondiale
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